

PATIENT REFERRAL DOCUMENT

LMU Klinikum Munich

Patient: Laura Schneider

Date of Birth: 29.09.1995

Date: 15.05.2026

Referring Physician: Dr. Andreas Braun

General Practice Neuhausen, Munich

REASON FOR REFERRAL:

Patient with known history of bipolar disorder type 1, currently managed on lithium, presenting with significant deterioration over the past three weeks. Reports markedly reduced sleep · averaging 3 to 4 hours per night. Increased goal-directed behaviour, pressured speech, and grandiose thinking observed by family. Patient has not taken lithium consistently for the past two weeks stating she does not need it. Family extremely concerned about escalating behaviour including spending large amounts of money and making impulsive decisions. No current suicidal ideation reported but patient has poor insight into current mental state.

MENTAL STATE EXAMINATION:

Appearance: Dressed brightly, slightly dishevelled, agitated

Behaviour: Restless, unable to sit still during consultation

Speech: Pressured, difficult to interrupt, loud

Mood: Elated, stated she feels "better than ever"

Affect: Elevated, labile

Thought form: Flight of ideas noted

Thought content: Grandiose · believes she has special abilities

Perceptions: No hallucinations reported

Cognition: Oriented to time, place, person

Insight: Very poor · denies being unwell

CURRENT MEDICATIONS:

Lithium carbonate 800mg at night · not taking consistently for 2 weeks

Quetiapine 50mg at night · states she stopped one week ago as "not needed"

Lamotrigine 100mg twice daily · reportedly still taking

PAST PSYCHIATRIC HISTORY:

Bipolar disorder type 1 · diagnosed 2018

Two previous manic episodes · 2018 and 2021

One hospitalisation · 2021, voluntary admission for manic episode

No previous suicide attempts

Depressive episodes managed as outpatient

PAST MEDICAL HISTORY:

Hypothyroidism · on levothyroxine 75mcg daily

No other medical conditions

INVESTIGATIONS:

Lithium level: 0.3 mmol/L · subtherapeutic, therapeutic range 0.6 to 1.0

TSH: 2.1 mIU/L · within normal range on current levothyroxine dose

Full blood count: Normal

Renal function: Normal · important given lithium use

Random glucose: 5.2 mmol/L

SOCIAL HISTORY:

Works as a graphic designer, currently on sick leave

Lives alone in apartment

Supportive family · parents involved and concerned

In a relationship · partner also concerned

No alcohol or substance use

RISK ASSESSMENT:

Current manic episode with poor medication adherence represents significant risk. Patient at risk of impulsive decisions, financial harm, and potential deterioration to more severe mania. No current suicidal ideation but insight is very poor. Family able to monitor closely at present but concerned about overnight safety.

CLINICAL IMPRESSION:

Active manic episode in context of known bipolar disorder type 1, complicated by non-adherence to mood stabilisers. Subtherapeutic lithium level confirmed.

Requires urgent psychiatric assessment for consideration of medication reinstatement under supervision, possible short-term crisis support, and assessment of need for informal or formal admission. Family support is present but patient requires specialist input urgently to prevent further deterioration.

Urgency: Semi-urgent · please assess within 48 hours.